Vickie Berry Medical Record Review and Clinical Assessment

Comprehensive clinical comparison and appointment preparation

Patient: Vickie Berry | **Date of birth:** March 1, 1953 | **Age:** 73

Review and family updates: October 10, 2026 | Updated with head CT, urine results and reported urgency and frequency

Prepared for: Vickie Berry, her family, and her treating clinicians

The current priorities are newly reported wheezing and breathlessness with very little activity, a recent reading change in the context of significant carotid disease, and a separate concern about attention and remembering recent information. These issues require clinical assessment. The records support several possible contributors but do not establish one diagnosis explaining all symptoms.

At the last family update, she remained in the emergency department. Wheezing started before that visit and improved with prescribed Airsupra. The supplied export was generated during the encounter. Additional urine results and the 2:35 PM head CT are now available; the completed emergency assessment and disposition are not included. This is a synthesis of records and family observations, with interpretations and questions identified for discussion with the treating team.

Clinical priorities

- **Reassess the new breathing limitation after treatment.** Airsupra benefit supports a possible reversible airway component. The CT shows small-airway inflammation, but the small reported abnormalities should not automatically be assumed to explain marked exercise intolerance. Current examination, respiratory rate, response duration and functional tolerance matter.
- **Examine the recent reading change.** The last supplied carotid CTA, performed August 20, 2025, reported 70% left internal carotid stenosis. A new reading or comprehension deficit requires attention to onset, neurological findings and whether current stroke/TIA evaluation is indicated. The October 10 noncontrast head CT showed no acute abnormality, but the recent reading change still needs a neurological examination; see the new head CT section.
- **Arrange structured cognitive and functional assessment.** Family observations concern following television characters, repetition and possibly failure to remember recently watched material. She recognizes familiar people in person. The records do not establish dementia or loss of familiar-face recognition.
- **Verify actual medication exposure.** The old Parkview list conflicts with the family list. Resolve trazodone, thyroid dose, statin, aspirin, missing prescription refills and unspecified dosing frequencies using bottles and pharmacy history.
- **Complete important follow-up.** Reassess cardiac function if symptoms warrant, clarify vertebral fracture risk despite mild DXA findings, obtain missing GI and neurovascular follow-up, and characterize CKD using nephrology records and urine albumin testing.

Urgent changes while in the ER: Notify staff immediately for breathing difficulty at rest, inability to speak full sentences, chest pain, fainting or new unusual sleepiness/confusion. Sudden new speech or reading difficulty, weakness, visual loss or imbalance also warrants immediate attention. Outside emergency care, call 911 for severe breathing difficulty or sudden stroke symptoms, including symptoms that resolve. [1], [2]

Current symptoms and family observations

- **Urinary symptoms:** Her son confirmed urgency and frequency after the urine reports were reviewed. Whether these are new or worse than baseline, and whether fever, pain or burning are present, remain unclarified.
- **Breathing:** The son reports that wheezing and shortness of breath with very little exertion are new. They began before the ER visit, and Airsupra helped. The extent and duration of improvement are unknown. The CT request recorded "SOB x months," which may describe an earlier complaint but should not obscure this new change in symptoms and function.
- **Reading:** Difficulty started recently and is most evident with longer/complex words or hard-to-read content on her phone. Familiar messages from her son are easier. The original packet quotes, "I see words that aren't even there." This requires clarification; misreading, visual distortion, impaired comprehension and hallucination are different phenomena and none has been established by examination.
- **Recognition and television:** She recognizes familiar people in person. The problem described is distinguishing similar-looking TV/movie characters and following who is who. The original packet's wording should not be treated as a confirmed inability to recognize familiar faces.
- **Recent memory:** Her son describes repeated stories and watching a show again within a few days. The original packet reports not recalling previously seen movies. Clarify whether she genuinely fails to remember watching, chooses to rewatch, or was not attentive the first time. The duration and progression of these behaviors are not established.
- **Baseline attention:** The son has wondered about longstanding ADD/ADHD. No diagnosis is supplied. Childhood and adult baseline attention should be distinguished from recent changes.
- **Environment:** A disconnected dryer vent into a crawlspace and moisture/mold concern are family-reported. The two photographs do not establish the dryer fuel type. Fuel and clinical exposure remain unresolved.

Records and evidence

- Additional MyChart results: Test Details 11 and 12 for the October 10 urine specimen; Test Details 13 for the noncontrast head CT at 2:35 PM.
- Six-page family-maintained emergency packet: vickie-er-packet (3).pdf.
- Complete clinical narrative in the October 10 archive: seven XML documents and the 272-page 1 of 1 - My Health Summary.PDF. Much of this is repeated historical material, rather than separate new encounters. The 59 result entries in the two principal summaries are identical; the 14 results in the 2016 hospitalization summary repeat those entries.
- Page references below refer to the 272-page health summary unless labeled **ER packet**.
- Recent Kokomo diagnoses, the serial 2021-2026 kidney values, home medications, environmental history, and October 8 calcium scan are chiefly available through the family packet; their underlying provider records are not all included.
- No raw radiology images, completed October 10 provider/discharge note, or current physical examination is available here. Missing from this export does not prove a test or follow-up never occurred.

Timeline of key events

- **2009:** Bone-density report described normal mineralization. Lumbar radiographs showed mild degeneration and aortic atherosclerotic calcification; left-foot imaging showed a small dorsal heel spur. (pp. 170-173.)
- **September 2012:** Chest radiograph after a motor vehicle accident showed no active cardiopulmonary disease; vertebral compression deformities and kyphosis were noted. (pp. 168-169.)
- **July 6, 2015:** Lumbar imaging showed progression of degenerative disease and an L1 compression fracture. The presenting history was two months of pain without a known injury. (pp. 165-166.)

- **March 17-18, 2016: Hospitalization for acute neurological symptoms.** Sudden speech difficulty and predominantly documented right-sided weakness; treated with IV thrombolysis and transferred to Parkview ICU. Symptoms resolved. Neurology concluded a left middle-cerebral-artery-territory TIA; MRI and repeat CT showed no infarct or hemorrhage. Echocardiogram showed EF 60%, **grade I diastolic dysfunction**, and mild valve regurgitation. CTA reported a right carotid-siphon aneurysm measuring 4 x 3 mm and a left 3 x 2 mm aneurysm versus ulceration. Neuro-interventional consultation recommended surveillance without intervention. Clopidogrel and pantoprazole were started. (pp. 59-74, 225-246.)
- **July 18, 2017:** L1 fracture described as a stable **osteoporotic compression fracture**, with lumbar facet arthritis. (pp. 54-55.)
- **April 2018 and November 2019:** Left carotid ultrasound categorized narrowing as 50-69%; right was less than 50%. (pp. 50-53.)
- **February-September 2020:** Chest radiograph for cough was normal. Brain MRI was read as normal, and intracranial MRA showed no aneurysm or significant stenosis. Carotid ultrasound categorized both sides as less than 50%. An August echocardiogram entry has no usable interpretation. (pp. 42-51.)
- **April-August 2022:** Fall caused right distal fibular/lateral malleolar and proximal fifth-metatarsal fractures. Internal fixation was documented on subsequent radiographs; fluoroscopy occurred April 19, but the operative report is absent. Fractures were well healed by August 10. Head CT after a separate facial injury showed no acute intracranial pathology. (pp. 27, 33-41.)
- **July-September 2022:** DXA showed osteopenia, lowest T-score -1.3. CTA found no obvious intracranial aneurysm; the radiologist invited comparison with outside aneurysm images. Carotid ultrasound again categorized both sides as less than 50%. (pp. 25-32.)
- **September 26, 2023:** Left carotid ultrasound returned to the 50-69% category, described as increased from 2022; right remained less than 50%. (pp. 23-24.)
- **August 19, 2024:** Screening mammogram negative, BI-RADS 1. Earlier 2013, 2020 and 2022 mammograms were negative or benign. No cancer diagnosis or tumor-monitoring series is included. (pp. 22, 32-33, 49-50, 167-168.)
- **May 9, 2025:** Right knee radiographs showed moderate degenerative joint disease without fracture or effusion. (pp. 20-21.)
- **June 17, 2025:** Abdominal CT for cramping and rectal bleeding showed **sigmoid colitis**, small nonspecific pelvic fluid, a hiatal hernia, and aortoiliac atherosclerosis. The study did not establish the cause of colitis. (pp. 19-20.)
- **July 1, 2025:** Repeat DXA showed osteopenia, lowest T-score -1.1 at right total hip. Reported FRAX: major fracture 8.9%, hip fracture 1.1%; recommended repeat in 18-24 months. (pp. 17-18.)
- **August 20, 2025:** Head/neck CTA for left facial pain showed **70% left internal carotid narrowing**; right carotid narrowing was 0-25%. MRI showed no acute stroke, age-related atrophy and mild microvascular changes described as stable. Left maxillary sinus disease was present. No intracranial aneurysm was identified. (pp. 10-16.)
- **November 2025-May 2026:** Family packet records documentation of brain atrophy, autoimmune hypothyroidism, GERD, colitis, and later cough/constipation/diarrhea. These documentation dates are not necessarily dates of disease onset. Colitis was already visible on June 2025 CT. (ER packet pp. 2, 6.)
- **June-August 2026:** Family packet reports dyspnea, eGFR 50/creatinine 1.15 on June 2, CKD reclassified as stage 3a on August 20, and Wegovy initiation. Underlying prescribing and nephrology notes are absent. (ER packet pp. 2-6.)
- **October 7-8, 2026:** Family reports disconnected dryer vent into crawl space and moisture/mold concern; new reading difficulty and television/movie recognition concerns as originally summarized; see the subsequent family clarification above. Coronary calcium score reportedly zero. The actual calcium-scan report is absent from the archive. (ER packet pp. 1, 5-6.)
- **October 10, 2026:** ED evaluation for worsening breathlessness/lightheadedness. BP 128/92, pulse 101, temperature 98.2 F, oxygen saturation 100%. WBC 15.5, absolute neutrophils 12.23, hemoglobin 13.9;

creatinine 1.17/eGFR 49; NT-proBNP 287; troponin T 7. Chest CTA excluded pulmonary embolism and showed small left-lung tree-in-bud opacities, moderate hiatal hernia, healed sternal fracture and chronic vertebral height loss. Export ends before the completed clinical assessment. (pp. 2-9.)
- **October 10, 2026 at 1:01 PM:** Urine collected during the ER visit. Mild pyuria, trace leukocyte esterase and trace bacteria were reported at 1:31 PM; specimen sent for culture. No culture result is supplied. See the additional urine results section.
- **October 10, 2026 at 2:35 PM:** Noncontrast head CT for confusion/headache showed no acute intracranial abnormality and a small left maxillary sinus air-fluid level. No infarction, hemorrhage or mass was identified on CT. Compared with the July 20, 2022 CT. This is a new supplied result, rather than a repeat of the August 2025 MRI.

Clinical progression and interpretation

Neurological function and carotid disease

- The 2016 event resolved clinically; discharge neurological examination was normal and NIH stroke scale was zero. A lingering problem-list label of expressive disorder does not establish a persistent deficit.
- Recently reported reading difficulty represents a change that needs examination. The family subsequently clarified that familiar people are recognized in person, while similar-looking television characters are harder to distinguish. Repeated stories and difficulty recalling recently watched shows raise a separate concern about attention and retention of new information. The timing is unclear: the records do not establish whether all symptoms began together or whether some evolved over months.
- The most recent measured left carotid stenosis is substantially greater than the 2016 approximately 20% estimate and the 2020/2022 ultrasound categories. This warrants updated neurovascular assessment. The carotid measurements fluctuated over time and used different methods, so the trajectory is not a precise linear growth curve.
- Reading problems can involve language, visual processing, eyesight, or attention. Face-recognition difficulty may involve different brain networks; these reports do not localize every symptom to the left carotid territory.
- **Priorities to assess:** stroke/TIA first; also delirium from illness or medication exposure, visual impairment, and episodic seizures if the clinical pattern suggests them. If symptoms persist after acute causes are assessed, obtain formal cognitive and visual-processing evaluation.
- August 2025 age-related atrophy does **not** diagnose Alzheimer disease or another dementia. The 2020 MRI described no atrophy or microvascular changes, whereas 2025 called them stable; image comparison is needed to establish an actual trend. (pp. 15-16, 44-45, 213-224, 232-233.)
- If the stenosis is confirmed and judged responsible for recent symptoms, vascular treatment decisions may be time-sensitive. Neurology and vascular surgery should determine whether intervention is appropriate. [3]

New head CT from October 10

The newly supplied two-page MyChart report documents a **noncontrast head CT at 2:35 PM on October 10, 2026**, requested for confusion, with additional history of confusion and headache. Parker Suit MD interpreted it for FW Radiology, comparing it with the July 20, 2022 CT.

- **Reported result:** No acute intracranial abnormality. No acute infarction, hemorrhage or mass was identified; gray-white differentiation was maintained, and ventricles were normal in size and morphology. No gross calvarial fracture was reported.
- **Effect on the assessment:** This is reassuring for an acute abnormality visible on this CT. Current head imaging is now documented, replacing the earlier information gap. It does not explain the recent reading difficulty or repeated stories, and no completed neurological examination or ER disposition is supplied.

- **Remaining neurological question:** A normal noncontrast CT does not exclude a small or early ischemic stroke or a TIA. If the onset, continuing symptoms or examination raises concern for a focal neurological event, ask the treating team whether MRI with diffusion-weighted imaging and updated vascular assessment are indicated. MRI is more sensitive for small ischemic lesions; the decision depends on clinical findings and timing. [4]
- **Vascular and cognitive limits:** This was not a head/neck CTA and does not measure the previously reported carotid stenosis or formally reconcile the aneurysm history. Lack of an atrophy or microvascular description does not establish reversal of the August 2025 MRI findings. This report does not settle whether a cognitive disorder is present.
- **Sinus finding:** A small air-fluid level was reported in the left maxillary sinus. Left maxillary sinus disease was also described in August 2025, but these reports do not establish one persistent infection or a quantitative trend. Ask about nasal blockage/drainage, facial or dental pain, fever and symptom duration. Imaging alone does not distinguish bacterial from viral sinus inflammation or establish a need for antibiotics. This finding does not establish the cause of the cognitive or breathing symptoms. [5]

Attention and memory after family clarification

The family examples raise concern about registering and retaining new information, tracking a story, and reading unfamiliar material. Following a film requires attention to faces, names, scenes and plot. Difficulty distinguishing similar-looking characters does not identify one brain disorder. Reading familiar texts more easily may reflect the benefit of context; it does not establish normal language and attention with unfamiliar material.

Two mechanisms can appear as forgetfulness: information may not have been registered because attention was impaired, or it may have been registered but not retained. Medication effects, sleep problems, mood symptoms, thyroid dysfunction and illness can contribute. Formal testing of attention, learning, delayed recall and recognition can help distinguish these mechanisms. [6]

- **Cognitive impairment:** If repetition and loss of recent memories are increasing or represent a clear change from baseline, assess for mild cognitive impairment or dementia. Alzheimer disease, vascular contributions and mixed causes are possibilities to evaluate, rather than established diagnoses.
- **Daily independence:** Medication errors, bills, appointments, cooking, navigation and phone use are important. Mild cognitive impairment generally preserves everyday independence; dementia interferes with it. Recognizing familiar people can remain intact while other abilities are impaired. No current functional assessment is supplied. [7]
- **Acute worsening or fluctuation:** Assess delirium if thinking became worse over hours/days during illness or fluctuates, especially with impaired attention or unusual drowsiness. Acute illness and a longer-standing cognitive problem can coexist; the available history does not establish delirium. [8]
- **Lifelong attention:** ADHD begins in childhood. A longstanding attention pattern could coexist with another condition but would not adequately explain a genuinely new reading change in her seventies. [9]
- **Vision:** Current corrected vision and hearing still need assessment even when the family perceives a cognitive component. The old contradictory vision descriptions do not establish her current visual status.

After urgent causes and acute illness are addressed, arrange a dedicated cognitive visit with an informant who knows her baseline. Use a validated cognitive screen and functional history. If the brief screen does not explain the family concerns, request neuropsychological testing of attention, learning, delayed recall/recognition, language/reading, and visual processing. A normal screening score alone should not dismiss persistent symptoms. Testing during acute illness may underestimate baseline ability and require reassessment. [6]

Check current thyroid function, vitamin B12 and relevant metabolic results; review sleep, mood, sensory function and medications. No B12 deficiency or uncontrolled thyroid disease is established in these records. Record dated

examples from everyday life, including context, attention, whether reminders help, and impact on independence, rather than repeatedly quizzing her. [10]

Breathing and small airway inflammation

- Symptoms reportedly worsened despite months of breathlessness. October 10 CTA shows an objective small-airway abnormality, but no pulmonary embolism, consolidative pneumonia, pleural effusion, or enlarged thoracic lymph nodes.
- Neutrophilic leukocytosis supports assessing infection or inflammation, but can also reflect stress or recent systemic steroids. It does not identify an organism or establish sepsis.
- **Infection and aspiration are reasonable leading questions for the tree-in-bud pattern.** GERD plus moderate hiatal hernia makes recurrent reflux/aspiration worth investigating, particularly with cough after eating, nocturnal choking, regurgitation or swallowing difficulty. This connection is a hypothesis, not a confirmed diagnosis. [11]
- Wegovy was reportedly added after cough/dyspnea were already documented, so it cannot explain their original onset. Its delayed gastric emptying and GI effects could contribute to later symptoms in some patients; review whether nausea, vomiting, reflux, poor intake or dizziness changed with initiation/titration. [12]
- Lisinopril can contribute to chronic dry cough. That possibility does not explain the lung CT abnormality or new neurological symptoms by itself. [13]
- The reported clear lungs on October 8 cannot prove a new abnormality developed within two days: the original scan is missing and calcium scans and chest CTAs differ in lung coverage and technique. No direct comparison was reported.

New wheezing and breathlessness with little exertion

Reported benefit from Airsupra makes reversible airway narrowing a stronger possibility. Airsupra contains albuterol, which opens narrowed airways, and budesonide, which treats airway inflammation. This is a useful clinical clue, but an informal response does not prove asthma, measure obstruction severity, or exclude another contributor. [14]

Leading working possibilities: bronchospasm accompanying infectious or inflammatory bronchiolitis; reflux/aspiration-related airway irritation; underlying asthma or an irritant-triggered airway response. Her CT and blood count support investigating inflammation/infection, while GERD and hiatal hernia support asking about aspiration. None is confirmed by the current records. This is a ranking of issues to investigate, not a numerical probability estimate.

- Clarify symptoms with meals, lying down, regurgitation, nocturnal choking and swallowing. These would strengthen the reason for reflux or swallowing evaluation.
- Determine whether Airsupra benefit was substantial and sustained or whether breathlessness quickly returned; verify technique and the exact prescribed plan.
- Obtain current respiratory rate, breathing effort, air entry/wheeze and vital signs. If the ER team considers it safe, supervised assessment during a short walk can characterize symptoms, pulse and oxygenation. She should not be asked to perform an unsupervised exertion test.
- A recorded oxygen saturation of 100% does not measure airflow or establish normal exertional tolerance. Whether oxygen was being administered is unspecified.
- The CT excludes PE at the time of the study and reports no consolidative pneumonia, pneumothorax or pleural effusion. The airway findings mean the scan is not entirely normal; their limited reported extent should not automatically explain severe functional symptoms.
- Decide clinically whether treatment beyond the existing inhaler is needed and whether infection testing or antibiotics are appropriate. WBC alone does not identify a bacterial infection.

- After acute symptoms are stabilized, spirometry with bronchodilator testing can assess airflow obstruction and reversibility. Confirm asthma with compatible history and objective testing. [15], [16]

Heart-related breathlessness, arrhythmia or another contributor should remain under consideration if symptoms persist despite airway improvement. Heart failure can cause wheezing, but the CT does not describe pulmonary edema or pleural fluid and NT-proBNP 287 is below the commonly used acute exclusion threshold of 300, making acute decompensated heart failure less likely. The existing ten-year-old echo does not establish present cardiac function. [17], [18]

Because the wheezing preceded the ER visit, today's contrast administration is not the initial trigger described by the family. The degree of recovery after treatment should inform the treating team's disposition decision and follow-up plan.

Heart function and cardiovascular risk

Only one current troponin result is included, and the current ECG/telemetry interpretation is unavailable. A single normal troponin does not constitute a complete assessment of every cardiac cause. New orthopnea, leg swelling, rapid weight gain, chest discomfort or palpitations would change the clinical assessment.

- The 2016 echocardiogram was summarized as normal, but its detailed report recorded grade I diastolic dysfunction, meaning mildly impaired relaxation, despite EF 60%. This alone does not establish heart failure. (p. 66.)
- NT-proBNP 287 is above this laboratory's reference limit, but below a commonly used acute-heart-failure exclusion threshold of 300. It lowers the likelihood of acute decompensated heart failure while not settling possible chronic heart failure with preserved EF. Age, kidney function and clinical context affect interpretation. A current echo is more informative than the ten-year-old result. [19]
- A reported coronary calcium score of zero is reassuring about calcified coronary plaque, but does not erase carotid/aortic disease or exclude every cardiac cause of dyspnea. [20]

Kidney function and metabolic status

- Family-reported eGFR: **72 (2021), 54 (Jan 2023), 49 (Nov 2023), 46 (Mar 2024), 42 (Oct 2024), 53 (Jan 2025), 50 (Jun 2026), 49 (Oct 2026)**.
- Current creatinine 1.17 is close to the June value 1.15. Current eGFR fits stage 3a; the earlier 42 fits stage 3b. That staging difference can reflect changing kidney function rather than a contradiction.
- These data show longer-term loss of kidney function with fluctuation and recent stability, not clear evidence of an acute October decline. The October sample preceded chest-CTA contrast, so it is not a post-contrast result.
- Normal BUN 13, sodium 140, glucose 106, calcium 9.4, bicarbonate 23, and hemoglobin 13.9 do not support advanced uremia, severe metabolic disturbance or anemia as the main explanation for the acute neurological change. This is an inference from the supplied results, not a diagnosis of its cause. (p. 8; ER packet p. 5.)
- Urine albumin/creatinine ratio and nephrology notes are needed for fuller CKD risk assessment. Historical eGFR values may use different calculation methods.

Bone density and vertebral fracture risk

- Right ankle/foot fractures healed by August 2022. The 2025 DXA report describes no significant change from 2022, with osteopenia rather than a T-score diagnosis of osteoporosis.
- **The L1 compression fracture is an important separate finding.** It was present by 2015 and explicitly called osteoporotic in 2017. October 2026 CT again shows chronic vertebral height loss.
- The 2025 DXA report's discussion of FRAX thresholds lists absence of prior hip/vertebral fracture as a condition for that approach. If L1 is confirmed as a fragility fracture, the report's low FRAX score and "no medication threshold" conclusion should be reconsidered in that context. Guidelines support treatment consideration after vertebral fracture regardless of BMD. [21]

- The 2022 scan used **GE Lunar** and 2025 used **Hologic**. The "previous" BMD numbers quoted in 2025 differ from the raw 2022 report. They may have been converted or calibrated, but that is not documented. Cross-calibration is needed for valid quantitative comparison; ask the imaging facility to verify it and whether fractured L1/degenerative levels were appropriately handled. [22]
- The repeat DXA was completed, so it is not currently missing as the emergency packet suggests. The report's next interval would be approximately January-July 2027, subject to clinical reassessment.

Gastrointestinal and other findings

- Sigmoid colitis was demonstrated in June 2025; its cause and resolution are not established. Ischemic, infectious and inflammatory causes require the actual GI evaluation; atherosclerosis alone does not identify the cause.
- Hiatal hernia was present in June 2025 and remains moderate on October 2026 chest CT; no reliable enlargement trend is established.
- Left maxillary sinus abnormalities appear in 2022 and 2025; follow-up and symptom burden are unclear.
- Mild alkaline phosphatase elevation (123) and direct bilirubin (0.3) merit context/recheck if persistent, but the supplied results do not establish significant liver disease. Glucose 106 alone does not diagnose diabetes.
- No documented malignancy, enlarging tumor or cancer-treatment course is present in these files.

Gout is referenced in the family packet, but diagnostic details, uric acid results and treatment are not supplied. The available records do not establish its current activity.

Current laboratory and vital sign findings

The following selected results were collected October 10, 2026 at 11:27 AM EDT. Values and laboratory ranges are from the compiled health summary, pp. 4-8. Interpretation is contextual; an abnormal flag is not a diagnosis.

Test	Result	Laboratory range	Clinical relevance
WBC	15.5 x 10^3/uL	3.4-10.5	Elevated; assess infection, inflammation, stress and recent steroids.
Absolute neutrophils	12.23 x 10^3/uL	1.60-6.50	Elevated; does not identify an organism.
Hemoglobin	13.9 g/dL	12.0-15.5	No current anemia to explain breathlessness.
Platelets	283 x 10^3/uL	150-450	Within range.
NT-proBNP	287 pg/mL	0-125	Above lab range; below common acute HF exclusion threshold of 300.
Troponin T Gen 5	7 ng/L	0-10	Within range; only one result included.
Creatinine	1.17 mg/dL	0.60-1.10	Close to June family-reported 1.15; no clear acute rise.
eGFR	49 mL/min/1.73 m2	Greater than 59	Current stage 3a range; calculation is not identical to drug-dosing clearance.
BUN	13 mg/dL	7-18	Within range; does not support advanced uremia.
Sodium and potassium	140 and 3.9 mmol/L	134-146 and 3.6-5.1	Within range.
Chloride and CO2	105 and 23 mmol/L	98-107 and 21-29	Within range; CO2 here reflects serum bicarbonate, not a respiratory blood gas.

Test	Result	Laboratory range	Clinical relevance
Anion gap	12.0 mmol/L	9.0-16.0	Within range.
Calcium	9.4 mg/dL	8.5-10.3	Within range.
Glucose	106 mg/dL	70-99	Mild flag; fasting status unknown and does not diagnose diabetes.
Albumin and AST	4.3 g/dL and 19 U/L	3.4-5.0 and 10-35	Within range.
ALT	9 U/L	10-35	Slightly low; no major liver injury pattern established.
Alkaline phosphatase	123 U/L	40-110	Mildly elevated; interpret/recheck in context if persistent.
Total and direct bilirubin	0.6 and 0.3 mg/dL	0.0-1.0 and 0.0-0.2	Total normal; direct minimally elevated.

Relative lymphocytes were 15.4%, below the laboratory percentage range, but the absolute count was normal at 2.39 x 10^3/uL. Absolute eosinophils were 0.03 x 10^3/uL. The CBC does not establish absolute lymphopenia or exclude asthma.

Recorded vitals around 11 AM were BP 128/92, pulse 101, temperature 98.2 F and oxygen saturation 100%. Respiratory rate and oxygen administration were unspecified. A single set of vitals does not establish current status or exertional tolerance. The blood draw preceded the 12:19 PM chest CTA, so these kidney values are not a post-contrast measurement.

The troponin and NT-proBNP reports note possible falsely decreased results with high-dose biotin supplements above 5 mg/day. The family lists a multivitamin, but no high-dose biotin exposure is established. Verify supplement doses if the treating clinician considers this relevant; do not assume these results are inaccurate.

Additional urine results from October 10

Two newly supplied MyChart reports describe the same clean-catch midstream urine collected October 10, 2026 at 1:01 PM, with urinalysis and microscopy finalized at 1:31 PM. Ordering clinician: Eric Douglas Wasilowski DO. The report states that the specimen was sent to microbiology for culture. No organism, colony count or antibiotic susceptibility result is supplied. The "Final" status applies to the reported urinalysis, not a completed culture result.

Findings

- **Urine white blood cells:** 6-10 per high-power field, mildly above the laboratory range of 0-5.
- **Leukocyte esterase:** Trace, flagged abnormal. Together with the WBC count, this supports mild urinary inflammation (pyuria).
- **Bacteria:** "1+ Trace," flagged abnormal. This microscopic finding does not identify an organism or distinguish symptomatic infection from bacteria present without symptoms or contamination during collection.
- **Nitrite:** Negative. This does not exclude UTI because some bacteria do not produce nitrite. [23]
- **Red cells and epithelial cells:** RBC 0-2 per high-power field and epithelial cells 0-4 per low-power field, within their stated ranges. A normal epithelial-cell count does not settle the cause of the bacteria finding.
- **Specific gravity:** At or below 1.005, compared with a stated range of 1.005-1.030. The sample is dilute. Fluid intake, possible IV fluids and collection context should be considered; a single dilute sample does not establish worsening CKD or a separate concentrating disorder. No fluid administration history is supplied. [24]

- **Other chemistry:** Protein, blood, glucose, ketones and bilirubin were negative. Urine was yellow and clear; pH 6.0 and urobilinogen 2.0 mg/dL were within the stated ranges.

Interpretation and effect on the assessment

Symptomatic UTI is now a stronger working possibility. Her son subsequently confirmed urinary urgency and frequency. Their onset and whether they are new or worse than baseline remain unknown; burning, suprapubic/flank pain, fever and rigors have not been clarified. Urgency and frequency are symptoms compatible with a bladder infection. If new or worsened, their combination with the urine inflammation and bacteria increases concern for symptomatic infection. The treating clinician should assess the current history and examination, with culture follow-up; a confirmed diagnosis and organism are not supplied. [25], [26]

If an infection is confirmed and cognition has acutely worsened or fluctuates, illness-related delirium remains a consideration. These mild urine abnormalities do not establish a urinary cause of the recent reading change, repeated stories, elevated blood WBC count or respiratory symptoms. The neurological and breathing assessments still need to proceed on their own clinical findings.

The earlier caution about confusion with bacteria in the urine but no local urinary symptoms needs to be applied to the clarified history. Urgency and frequency are now reported, so urinary symptoms should not be described as absent or undocumented. The ER team should determine whether these are acute infection-related symptoms and whether treatment while culture is pending is appropriate. Chronic noninfectious urinary symptoms can coexist with incidental bacteriuria, so the timing still matters. The urine findings alone do not establish that infection caused her cognitive or respiratory problems. [27]

The negative urine-protein dipstick does not replace a quantitative urine albumin-to-creatinine ratio for CKD assessment, especially in a dilute specimen. The previously identified urine-albumin information gap remains. [28]

Follow up questions

- "She has urgency and frequency. Are these new or worse than baseline, and do her examination and urine findings support a symptomatic UTI requiring treatment now?"
- "Who will review the pending culture after discharge, and how will we receive the result?"
- "Does the culture show a predominant organism, a significant colony count or mixed growth, and are susceptibilities reported?"
- "If an antibiotic is prescribed, what infection is it treating, and is the choice and dose appropriate for her kidney function?"

No new medication order or antibiotic administration is documented in these two reports.

Medication history and reconciliation

Historical prescription status is not proof of current use. The five medicines labeled active in the 2026 Parkview summary match the continuing-medication portion of the March 2016 discharge. Carry-forward is plausible; bottle labels, dispensing history and prescriber reconciliation should determine the current list.

Medication	Documented history and current conflict
Trazodone	2016 home/discharge list: 600 mg at bedtime; inpatient order: 300 mg as needed. Parkview still lists 600 mg nightly. Family home list: 50 mg nightly. Actual current dose and change date unknown.
Venlafaxine XR	150 mg daily in 2016 and both 2026 lists. Family packet reports no refills remaining; actual missed doses are unknown.

Medication	Documented history and current conflict
Citalopram	Historical 2016 list recorded 80 mg daily; explicitly stopped at discharge. Not on the current home list.
Clopidogrel	Started at 75 mg daily March 18, 2016. Current home list includes it. Historical order is labeled expired in Parkview; this does not prove current therapy stopped.
Aspirin	81 mg daily continued alongside clopidogrel in 2016. Still in Parkview active list; absent from family home list. Later intended antiplatelet plan is missing.
Statin	Pravastatin 10 mg daily in 2016 and Parkview; family reports replacement by atorvastatin 40 mg nightly. Switch date unavailable.
Levothyroxine	112 mcg daily in 2016/Parkview versus 100 mcg daily at home. Dose-change date and current TSH/free T4 unavailable.
Lisinopril	40 mg daily before and after 2016 discharge and on current home list. Old order stopped/reissued with the same stated dose; no clear actual dose change established.
Amlodipine/clonidine	Home list: amlodipine 10 mg daily, clonidine 0.1 mg with frequency unspecified. Addition dates absent.
Pantoprazole/ omeprazole	Omeprazole 20 mg stopped in March 2016; pantoprazole 40 mg started because of the clopidogrel interaction concern. Pantoprazole remains on home list; frequency needs verification.
Buspirone	Home list includes 10 mg tablets; frequency, start date and titration unavailable.
Inhalers	Family reports standalone albuterol HFA not used since October 2026. Airsupra remains as needed and itself contains albuterol plus budesonide; current rescue plan needs clarification.
Wegovy	Family reports initiation August 20, 2026; current 0.5 mg pen. Initiation/titration orders and injection schedule absent; packet reports refill issue.
Anti-inflammatory drugs	Naproxen 500 mg twice daily stopped in 2016. Family reports ibuprofen discontinued later because of CKD.
Other prior BP drugs	Hydrochlorothiazide and olmesartan reportedly no longer taken; dates/reasons absent.
Supplements/MiraLAX	Daily iron, multivitamin, calcium and MiraLAX listed at home. Exact doses, indication testing and timing unavailable.

Medication issues requiring review

- **Resolve the 600-versus-50 mg trazodone discrepancy before anyone relies on the exported dose.** The product label generally limits outpatient dosing to 400 mg/day in divided doses; 600 mg/day is an inpatient maximum in divided doses. Sedation, low standing BP and impaired thinking are recognized risks. This record does not prove she is taking 600 mg or has drug toxicity. [29]
- Request review of venlafaxine dosing for her renal function, the combined trazodone/venlafaxine/buspirone regimen, and any recent missed doses or additions. This combination warrants checking sedation, orthostasis and serotonergic effects; no documented fever, clonus or hyperreflexia establishes serotonin syndrome here. [30]
- Check seated/standing BP and actual clonidine schedule. One seated BP does not assess medication-related postural lightheadedness.

- Verify whether aspirin was intentionally stopped; the 2016 dual-antiplatelet instruction is not a current treatment plan. The GI bleeding history also matters in that decision.
- Review thyroid dose, TSH/free T4, and how levothyroxine is timed around iron/calcium.
- Have the treating clinician reconcile changes; this comparison is not a prescription to start, stop or change doses.

Additional medication clarifications

- Venlafaxine labeling recommends reducing the total daily dose by 25-50% for mild/moderate renal impairment, with individualization. Her eGFR is not identical to the creatinine clearance used in that recommendation, and prior dose adjustments are unknown. Ask the prescriber to assess the existing 150 mg XR regimen; do not assume that dose is definitively inappropriate or stop it abruptly. Missed doses are a relevant question because the family packet reports a refill issue. [30]
- Long-term dual antiplatelet therapy is generally not recommended after stroke/TIA; short-term combinations are used in selected situations. Clopidogrel alone may be an intentional plan. Absence of aspirin from the family list is not itself proof of inadequate prevention, and aspirin should not be added independently. The prior rectal bleeding history matters. [31]
- Airsupra itself contains albuterol; not using a separate albuterol HFA inhaler does not mean she has no albuterol exposure. Verify inhaler technique, frequency, response and the written rescue plan with the ER team. Persisting or worsening symptoms despite treatment require reassessment. [14]
- Ask about recent steroids, antibiotics, new prescriptions, OTC sleep/allergy medicines and actual missed doses. Zero refills in a dashboard does not establish that medication has run out.
- Do not abruptly stop clonidine or venlafaxine based on this document. Reconciliation and any dose changes belong with the treating clinician/pharmacist.

Discrepancies and corrections

- **New head imaging:** The October 10 noncontrast CT fills the previous current-imaging gap. Its absence of an atrophy/microvascular description does not contradict or reverse the August 2025 MRI findings; the two reports use different modalities and emphasis.
- **Family symptom clarification:** Familiar people are recognized in person. Television-character confusion should not be documented as confirmed loss of familiar-face recognition. The new reading issue and the less clearly dated memory symptoms need separate timelines.
- **New respiratory change:** The CT request's "SOB x months" does not capture the son's later description of new wheezing and marked breathlessness with light activity. Document both the older history and this reported new change.
- **Aneurysm history:** 2016 CTA and neuro-interventional consultation documented small bilateral carotid-siphon aneurysms/dysplastic changes. MRA 2020, CTA 2022 and CTA 2025 reported no aneurysm. The 2022 statement that prior imaging showed none conflicts with the 2016 report. Obtain side-by-side image review; disappearance cannot be established from wording alone. One copied 2016 problem-list note also says "no saccular aneurysm," whereas its full H&P and specialist assessment say an aneurysm was present. (pp. 28-29, 46, 70-74, 237-243.)
- **Stroke versus TIA:** Administrative problem lists say stroke/CVA and expressive disorder; the treating 2016 neurologist concluded TIA with full recovery and negative infarct imaging. Later imaging histories mention a 2014 stroke, but the supplied detailed hospitalization occurred in 2016. A separate 2014 event is unverified. (pp. 44-46, 225-233, 246.)
- **2016 symptom side:** Several original imaging histories say left-sided weakness, while most clinical descriptions document right-sided weakness and aphasia. The ED physician recognized this discrepancy. (pp. 69-74, 246-248.)

- **Carotid percentages:** Left 50-69% in 2018/2019, less than 50% in 2020/2022, 50-69% in 2023 and 70% on 2025 CTA. Consider measurement variation alongside possible disease change. "No intracranial flow-limiting stenosis" in the 2025 impression refers to a different arterial location and does not negate the neck-carotid 70% finding.
- **Brain atrophy date/trend:** November 2025 in the family packet is a documentation date; an August 2025 MRI already mentions atrophy. Its "stable" wording is not a measurable comparison with the 2020 report that called the MRI normal.
- **Colitis timing:** Family summary places notation/diagnosis in November-December 2025; CT already showed colitis in June 2025. This may reflect later diagnosis entry rather than a new episode.
- **Bone follow-up:** Packet says follow-up after 2022 is overdue; actual July 2025 scan is included. Conversely, that scan's risk interpretation may need reconsideration because of the older vertebral fracture.
- **Medication conflicts:** Trazodone, thyroid dose, statin and aspirin differ; several old orders marked expired are currently reported at home. These are unresolved medication-reconciliation issues, not verified additions/discontinuations in October 2026.
- **Vision:** 2016 therapy notes use "legally blind" while documenting reading with contact lenses, independent function and driving; nursing screens deny serious difficulty with corrected vision and mention cataracts. Obtain current best-corrected vision and clarify the historical term. Old screening responses are not a current eye examination. (pp. 201-202, 212, 215, 230, 234.)
- **Care team/data quality:** Parkview lists Dr. Akande as PCP, while the family lists Dr. Samonte. Some screening dates are clearly administrative placeholders. A 2026 import/signature header does not make the embedded 2025 examination a 2026 test.
- **Allergies:** Parkview consistently records no known active allergies; the family packet contains no separate verified allergy list. No actual allergy contradiction can be established.

Missing information and questions for clinicians

Urgent neurological questions

- "Has her recent reading difficulty been examined, and does its timing and pattern warrant stroke/TIA assessment? What was her last-known-normal time? She recognizes familiar people in person; the television issue concerns similar-looking characters."
- "What does the neurological examination show, including visual fields, reading, naming, comprehension and recognition? The 2:35 PM noncontrast CT showed no acute abnormality. Does her history or examination still warrant MRI with diffusion imaging or vascular imaging?"
- "The August 2025 CTA reported 70% left carotid stenosis. What is the current degree, could these symptoms make it symptomatic, and has vascular surgery reviewed it?"
- "If current imaging is unrevealing but symptoms persist or are episodic, what is the plan for seizure, cognitive and ophthalmic evaluation?"

Breathing and heart questions

- "The wheezing and breathlessness with very little activity are new, and Airsupra helped. What does her examination show now, and is the improvement sustained?"
- "If you consider it safe, can you assess symptoms, pulse and oxygen during a short supervised walk before deciding on discharge?"
- "Does she need additional bronchospasm treatment, and what is the exact inhaler plan and return advice?"
- "When should she have reassessment and spirometry after the acute episode?"
- "Does the combination of WBC 15.5 and tree-in-bud opacities indicate infection, aspiration or another bronchiolar process? What treatment and reassessment are planned?"

- "Could reflux from the hiatal hernia cause recurrent aspiration? Do symptoms warrant swallowing or reflux evaluation?"
- "What were the ECG/telemetry results? Should persistent dyspnea prompt an updated echo, lung-function testing and oxygen assessment during walking?"
- "If the CT changes persist, is pulmonary consultation or sputum testing indicated?" Persistent findings may justify cultures, including mycobacterial testing when the clinical pattern supports it; the present scan alone does not diagnose that infection.
- "Has she recently used oral steroids, antibiotics or frequent rescue inhaler doses that could affect WBC or pulse?"

Environmental uncertainty

- **Dryer fuel type remains unknown.** If combustion exhaust exposure is independently suspected, discuss **carbon monoxide** promptly. CO can cause breathlessness, dizziness and cognitive changes; ordinary pulse oximetry can appear reassuring despite exposure. Blood carboxyhemoglobin testing is time-sensitive and may become less informative after time away from the source or oxygen treatment. If active exposure is suspected, leave the building and call emergency services. This is conditional; there is no documented CO exposure or level in these files. [32]
- Moisture/mold may contribute to cough or wheeze, but these records do not establish it as the cause of her neurological symptoms. Repairing the moisture/vent problem matters regardless of mold species; commercial urine mycotoxin testing does not establish a clinical diagnosis. [33], [34]

Medication kidney thyroid and bone questions

- "Can one clinician/pharmacist produce a verified medication list from actual bottles and pharmacy fills, including exact frequency, recent dose changes and missed doses?"
- "Is venlafaxine dosing appropriate for her calculated renal clearance, and could sedating drugs or postural BP changes contribute?"
- "What are her latest TSH/free T4, B12, urine albumin/creatinine ratio and any relevant iron studies? Which additional tests are indicated by her examination?"
- "Was the L1 fracture a fragility fracture? Does she need osteoporosis treatment assessment despite a T-score of -1.1, and were the DXA machines cross-calibrated?"
- "What established the cause and resolution of sigmoid colitis? Were colonoscopy/biopsy or stool studies done after rectal bleeding?"
- "Were the 2016 aneurysm images ever formally reconciled with the later negative studies, and is surveillance still needed?"

Cognitive assessment questions

- "Is the problem mainly attention/encoding, retention of recent information, language/reading, visual processing, or a combination?"
- "Have changes in medication use, bills, appointments, cooking, navigation and phone use been assessed?"
- "If the screening score is normal but the family examples persist, will she receive neuropsychological testing or a memory-clinic referral?"
- "Could current illness or medication effects worsen an underlying problem, and when should testing be repeated after recovery?"

Records to obtain

- Completed October 10 neurological examination and clinician interpretation of the negative CT; any subsequent MRI or vascular study if ordered, and the final ER assessment/disposition. Current noncontrast head CT is now supplied.

- Final urine culture from the October 10 1:01 PM specimen, including organism/colony count, susceptibilities if reported, the treating clinician interpretation and any antibiotic orders.
- Completed October 10 ED provider note, neurological findings, ECG/telemetry interpretation, repeat tests, medication administrations, discharge/admission decision and follow-up plan.
- Kokomo primary care, nephrology, GI, neurology/vascular and ophthalmology records; current pharmacy dispensing history.
- Original October 8 calcium-scan report/images and any later chest studies.
- Original 2016 and 2020/2022/2025 neurovascular images for direct comparison.
- Original DXA calibration/comparison information, vertebral fracture history and any osteoporosis treatment records.
- Follow-up mammography after August 2024 and actual colonoscopy history. A screening colonoscopy is mentioned in the 2016 history, but its date/result are absent; the automated "due since birth" field is not reliable evidence of no screening.

Care team and coordination

The family identifies Dr. Hannah Cher Samonte and Heather J. Bye NP at Community Physician Network in Kokomo for primary care, and Dr. Melissa Anderson for nephrology. Parkview lists Dr. Olusina Akande as PCP. Confirm the current coordinator and send the final emergency assessment and reconciled medication list to that clinician. Office contact details should be verified before use; the two systems may not share the same current information.

Source record locations

- **New MyChart head CT report:** Test Details 13, two pages. October 10, 2026 at 2:35 PM; noncontrast CT for confusion/headache; no acute intracranial abnormality and small left maxillary sinus air-fluid level. Interpreted by Parker Suit MD; compared with July 20, 2022. This is the radiology report, with no accompanying raw scan or completed neurological examination.
- **New MyChart urine reports:** Test Details 12, two pages, for urinalysis with culture if indicated; Test Details 11, one page, for urine microscopy. Both describe the October 10 1:01 PM specimen and 1:31 PM results. They provide new urine findings but no culture result or completed ER assessment.
- **Family emergency packet:** Six pages, with home medications, family-reported chronology, Kokomo clinicians and initial symptom wording. Later family clarifications in this report supersede the broader wording about face recognition and establish new wheezing, pre-ER onset and reported Airsupra benefit.
- **Compiled health summary:** 272 pages from the October 10 archive. Current medications/vitals/chest/labs are on pp. 1-9; August 2025 CTA/MRI on pp. 10-16; 2025 DXA on pp. 17-18; colitis CT on pp. 19-20; carotid ultrasound history on pp. 23-26 and 42-53; 2022 DXA on pp. 30-31; 2017 L1 fracture on pp. 54-55; 2016 echo on pp. 62-66; 2016 CTA on pp. 71-74; detailed hospitalization on pp. 225-246.
- **Source interpretation:** Embedded examination dates are used instead of later import/signature dates. Repeated copies of the same test are not treated as separate clinical events. Radiology reports were reviewed; raw imaging was not available for direct interpretation. Follow-up absent from this export may have occurred elsewhere or after it was generated.

Clinical references

1. NHLBI emergency guidance. Open source
2. American Stroke Association. Open source
3. ESVS carotid guidelines. Open source
4. Canadian Stroke Best Practices brain and vascular imaging. Open source
5. AAO HNS adult sinusitis guideline 2025. Open source
6. Specialty cognitive diagnostic guideline. Open source
7. National Institute on Aging. Open source
8. NICE delirium guideline. Open source
9. NIMH adult ADHD guidance. Open source
10. Primary-care cognitive diagnostic guideline. Open source
11. Tree-in-bud study. Open source
12. Wegovy prescribing information. Open source
13. Lisinopril prescribing information. Open source
14. Airsupra prescribing information. Open source
15. NHLBI asthma diagnosis. Open source
16. NICE asthma guideline. Open source
17. Mayo Clinic cardiac wheezing guidance. Open source
18. ESC heart-failure guideline. Open source
19. ACC biomarker interpretation. Open source
20. ACC calcium-score review. Open source
21. BHOF clinician guide. Open source
22. ISCD positions. Open source
23. MedlinePlus nitrite interpretation. Open source
24. MedlinePlus urine specific gravity. Open source
25. NIDDK bladder infection symptoms. Open source
26. NIDDK bladder infection diagnosis. Open source
27. IDSA asymptomatic bacteriuria guideline. Open source
28. NIDDK UACR and GFR guidance. Open source
29. Trazodone prescribing information. Open source
30. Effexor XR prescribing information. Open source
31. AHA secondary-prevention guideline. Open source
32. CDC CO guidance. Open source
33. CDC mold guidance. Open source
34. CDC mycotoxin guidance. Open source